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NSC 4319 02
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MS Case Vignette

The case vignette written by me:

Female patient Mary Smith, age twenty-four, is presenting the following symptoms: blurred vision, pain behind the eyes, mood instability, balance difficulties, and muscle weakness. She describes some days as being better than others, and some worse. However, her day-to-day life has been disrupted by these complications: last week she fell off the beam at her gymnastics practice for their upcoming competition. She reports that this has happened twice previously, all occurring within the last month. The patient has not been diagnosed with any underlying diseases or disorders. Given my patient's age, gender, and reported symptoms, my hypothesis would be that Mary is experiencing early-stage onset of the disease known as "Multiple Sclerosis."

The case vignette generated by AI:

Elena, a 32-year-old female, presents to her physician reporting a three-week history of numbness and a "pins and needles" sensation in her lower extremities that has recently progressed to her mid-thighs. She also describes experiencing bouts of double vision and a profound, overwhelming fatigue that has significantly impacted her ability to perform her daily duties as a primary school teacher. Elena recalls a similar, though less severe, episode of unexplained dizziness and unsteady gait approximately eighteen months ago that lasted for several weeks before resolving on its own. Upon clinical examination, she displays mild weakness in her left leg and hyperreflexia,

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suggesting involvement of the central nervous system over multiple points in time and space, which is a hallmark of this disorder.

Comparison narrative:

Comparing my vignette to AI's, the first similarity I noticed were the basics: same gender (female), same age range (both fit in 20-40 range), and both having vision problems. These are hallmark signs of Multiple Sclerosis. Secondly, I noticed the duration of symptoms were similar as well: I mentioned that my patient had experienced 3 specific events within 4 weeks, whereas the AI mentioned that these symptoms had persisted over the previous 3 weeks (only a week difference between our vignettes). Third, both the AI and I ensured that we mentioned a disruption to the "daily life" of our patients. As for differences, I noticed the AI mentioned a progression/worsening of symptoms, something that I did not find important to mention. However, this is probably a good detail to include because it clues in the fact that it's likely to be MS (since it almost always spreads as myelin degeneration does not stay localized).

As for unlikely or inaccurate aspects of the AI's vignette, I could only think of one specific issue: the fact that the patient experienced a disruptive episode 18 months ago that lasted for weeks! The symptoms disappeared on their own not long after, but have recently resurfaced, a year and a half later. Eighteen months is a long time! Can an individual have a period of remission that long without undergoing treatment? She never thought to go see a physician at the time, or ever since then? Especially if she was experiencing "overwhelming fatigue" that interrupted her ability to teach/work, and the

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pain had spread in her body, I would think she would be distressed and seek immediate medical attention. The lack of urgency honestly makes me think that the issue would be something other than MS...seems misleading/confusing to include this (I probably would not have included this myself). That being said, AI did a great job overall and it was quite similar to mine (it just picked a different "affected site"). There was nothing missing from its work either! I was pleasantly surprised!